Neodent Dental Hospitals: Treatment Page Research Pack

Oct 24, 2026 · @Ravi-Master

Executive summary

This pack gives a writer and a Neodent dentist a sourced starting point for seven treatment pages, and it cannot yet support any clinic-specific statement. The two clinic locations in the brief were unfilled placeholders, and the clinic's own website could not be opened from this workspace, so every location, clinician, service, contact and hours detail is unverified.

What the research found

Treatment	Evidence quality	Main message for the page	Biggest gap
Composite build-up	Good (ADA, university leaflet)	Lifespan is variable; do not promise years	Clinic's meaning of "build-up"
Dental implants	Good for process and risks; figures vary widely	Staged, long-term care; quote no success rate	Current peri-implantitis and success data; Indian guidance
Root canal treatment	Moderate (Mayo 2025, NHS Lothian 2023)	Calm, proportionate; the tooth needs restoring afterwards	Dental professional body source
Braces and orthodontics	Moderate; several leaflets are old or undated	Commitment, retainers and daily care	Clear aligners and orthodontic records
Smile design	Moderate for veneers and whitening; no definition source	Planned options, irreversibility, no before-and-after photos	Definition; advertising rules (primary source)
Full mouth rehabilitation	Weak (directory, provider page, 2016 review)	Phased plan; keep claims modest	A guideline or textbook source

Treatment	Evidence quality	Main message for the page	Biggest gap
Scaling and polishing	Good (Cochrane 2018 and reviews)	Do not say everyone needs a clean every six months	Practice-specific recall advice

Five things to act on first

1. **Supply the verified location details** (see the verification checklist). Without them, location wording cannot be written.
2. **Get a dentist's review of every page.** Each treatment section ends with clinician-review flags.
3. **Resolve advertising rules before using photos, testimonials or "best" language.** A news report dated 15 Oct 2026 says India's National Dental Commission has cautioned registered dentists on testimonials, before-and-after photos and misleading claims. Check the primary advisory.
4. **Avoid numbers that sources disagree on** (implant success, peri-implant disease rates, composite lifespan, braces duration).
5. **Replace thin sources** where flagged (root canal, aligners, full mouth rehabilitation).

Where sources disagree or are limited

- Peri-implantitis is estimated at about 15% of implants in a 2026 Abu Dhabi guideline and about one third of patients in a 2014 RCS briefing.
- Composite lifespan appears as 5 to 7 years (a university leaflet) and about 82% survival at 10 years for posterior composites (ADA).
- Routine scale and polish every six or twelve months showed little or no benefit for gum health in low-risk adults, but the evidence covers two UK trials and does not apply to people with gum disease.
- Many patient leaflets are undated or past their review date. Dates are shown beside each source.

Source-quality limits

Most sources are UK or US hospital, university and professional-body pages. Few are Indian. The Dental Council of India and Indian Dental Association were not found to publish patient pages for these treatments in the searches done here, so a Neodent dentist should add local guidance where it exists. The search was a single pass and does not replace a systematic review.

Patient-information note (for every page)

General information only. A website cannot determine what treatment an individual needs. Only an examination by a dentist can, and readers should follow their own dentist's instructions.

Facts Neodent must verify before publication

Nothing below could be confirmed from the brief. The two clinic entries in the brief were unfilled placeholders, and the clinic's site could not be opened from this workspace. Every item stays "unverified" until a Neodent staff member supplies it.

Clinic and contact details (per location)

- [ ] Exact clinic name as it appears on signage and registration
- [ ] Full postal address, locality and PIN code
- [ ] Phone number, WhatsApp number and email, and which is monitored
- [ ] Opening hours, including Sundays, holidays and emergency cover
- [ ] Google Business Profile name, address and category matching the website exactly
- [ ] Map link, parking, accessibility and landmarks, if the page mentions them
- [ ] Whether appointments are by booking only, walk-in, or both

Services per location

- [ ] Which of the seven treatments each location provides (do not assume both provide all seven)
- [ ] Which treatments are done on site and which are referred out
- [ ] Equipment named on any page (for example CBCT scanner, microscope, intraoral scanner)
- [ ] Materials, brands and implant systems, if named

Clinicians

- [ ] Names, qualifications and registration numbers for any clinician named
- [ ] Which clinician works at which location and on which days
- [ ] Specialist titles (orthodontist, endodontist, prosthodontist, periodontist) only if the registered qualification supports them
- [ ] Permission to name and photograph each clinician

Claims and compliance

- [ ] Any price, package, finance offer or discount (none are included in this pack)
- [ ] Any success rate, timeline, "painless", "same day" or guarantee wording
- [ ] Before-and-after photos, testimonials and reviews. India's National Dental Commission advisory of 14 Oct 2026 cautions registered dentists against publishing these (see Smile design section). Check the primary advisory.
- [ ] Consent and privacy wording for any patient image
- [ ] A named dentist who reviews and signs off each page, and a review date on each page

Clinical statements the dentist should confirm

- [ ] The definition of "composite build-up" and "smile design" at Neodent
- [ ] Typical visit counts and timelines for root canal, implants and braces
- [ ] Aftercare and urgent-contact wording, and the emergency number
- [ ] Whether clear aligners, ceramic braces, veneers or whitening are offered

Local search: referring to the two locations accurately

This section is editorial guidance based on the brief's rules, not a sourced finding. No search volume, ranking, competitor or demand data was collected or should be implied.

Principles

- **One treatment page per treatment, with a short location block.** Keep the clinical content on one page and put each verified location in a labelled block ("Where to find us"), rather than copying the page for each locality.
- **Only name a location against a service Neodent has confirmed there.** If only one clinic does root canals, say so on that page.
- **Use verified details only.** Clinic name, address, phone and hours must match the verified record and the Google Business Profile exactly.
- **No unsupported superlatives or reach.** Avoid "best", "leading", "top", "near me", "affordable", "painless", "same-day", "24/7", and any "serving [area]" line, unless there is evidence for it. Dentist advertising is also under regulatory scrutiny in India (see Smile design section).
- **Do not name clinicians, equipment or prices** unless verified.

Wording patterns (fill in once verified)

Use	Pattern
Location block	"[Treatment] consultations are available at [verified clinic name], [verified locality], [city]. Call [verified number] to ask about appointments." Add only after confirming availability
Service list on a location page	"Treatments offered at this clinic: [confirmed list only]"
Page title idea	"[Treatment] in [locality], [city]
Intro line	"[Treatment] is … Your dentist at [clinic name] can explain whether it suits you after an examination."
When a service is at one location only	"This treatment is offered at [location]. If you visit [other location], ask about referral."

The brief's two location slots (name, locality, address, phone, URL) were not filled in, so locality-specific wording cannot be suggested yet. Once Neodent supplies them, add the locality once in the title and once in the location block, and avoid repeating it in every heading.

Keeping details consistent

- Each location needs its own page or section with its own address and hours, and the treatment pages should link to it.
- Show the same name, address and phone everywhere (website, Google Business Profile, directories).
- Add a "last reviewed" date and a reviewing dentist's name to each treatment page.
- Add a short line that website information cannot determine what treatment an individual needs.

Do not claim

- Local demand, "popular in [area]", competitor comparisons, rankings or review scores used as proof of quality.
- A service area, travel time or "near [landmark]" unless the landmark and distance are verified.
- Emergency or out-of-hours availability unless Neodent confirms.

1. Composite build-up

Page angle: a repair-and-rebuild page built around "what is being rebuilt, with what material, and how long will it hold". It should be shorter and more practical than the implant or rehabilitation pages.

What it is, in plain language

A composite build-up uses tooth-coloured resin, bonded to the tooth and hardened with a blue light, to replace missing tooth structure. The ADA describes composite fillings as tooth-coloured resin that can be used in front or back teeth (ADA MouthHealthy, no date shown). A University of Bristol Dental School leaflet lists decay, tooth wear, trauma, cracks and fractures as reasons for use (Bristol leaflet, undated). "Build-up" is a clinic and patient term, not a separate clinical category in these sources. The writer should explain it as a larger composite restoration and ask the dentist how Neodent uses the term (see flags).

Why a dentist may discuss it

- Decay that has been removed and leaves a gap to rebuild.
- A chipped, cracked or worn tooth.
- A tooth that needs more bulk before another restoration, only if the Neodent dentist confirms this use.

Symptoms such as sensitivity, a rough edge or food trapping do not confirm what is wrong. Only an examination can.

Options and practical differences

Material	What the sources say	Main trade-off
Resin composite	Tooth-coloured and bonds to the tooth. Fracture and secondary decay are the main reasons for failure.	Needs a dry field and careful layering, so it is technique-sensitive
Amalgam	Strong and wear-resistant	Not tooth-coloured
Glass ionomer	Releases fluoride, tooth-coloured	Lower fracture resistance, not suited to heavy-load areas
Inlay or onlay (lab-made)	Listed by Bristol as an alternative where extra reinforcement is needed	Needs more than one visit and removes more tooth

Source for the material rows: ADA, Materials for Direct Restorations (updated 16 Aug 2024) and the Bristol leaflet.

How suitability is assessed

The dentist examines the tooth, usually with X-rays, and checks how much healthy tooth remains, where the edges of the damage sit, and whether the tooth can be kept dry. Bristol notes composites are less suitable when the tooth is hard to keep dry, the cavity extends near or under the gum, or the cavity is very large. The right plan depends on an individual assessment.

Typical process (general information)

1. Local anaesthetic if needed.
2. A rubber dam may be placed to keep the tooth dry.
3. Decay or damaged material is removed and the tooth shaped.
4. A bonding agent and the composite are applied, hardened with a blue light, then shaped and polished.

Steps follow the Bristol leaflet. ADA notes composite is placed in thin increments (about 2 mm layers) because of curing depth and shrinkage. Most patients need one visit for a straightforward case, but no source here gives a visit count, so avoid stating one.

Afterwards

Numbness can last several hours, so avoid eating on the numb side and avoid hot drinks until feeling returns (Bristol). Some sensitivity is normal; Bristol says it should settle within about two weeks and to contact the clinician if it persists. Readers should follow their own dentist's instructions.

Risks and limits, kept proportionate

- Composites can chip, roughen or discolour over time (Bristol).
- ADA notes polymerisation shrinkage and cost as limits.
- Contact the clinic if the bite feels high, an edge feels sharp, the filling comes out, or sensitivity does not settle.
- On safety, the ADA states the low-level BPA exposure from composites and sealants "poses no known health threat". Present this as the ADA's position, not as a guarantee.

Durability and maintenance

Sources express this differently, so do not promise a lifespan. Bristol gives a typical lifespan of 5 to 7 years, sometimes 10 or more. The ADA page cites about 82% survival at 10 years for posterior composites and says patient behaviour is the most significant factor in longevity. Factors that can affect results include the size of the restoration, bite forces and grinding, and oral hygiene. Regular check-ups let the dentist monitor the edges.

Prevention habits

Brushing twice daily with fluoride toothpaste, cleaning between teeth daily, a balanced diet and regular check-ups (ADA, Bristol). Not all damage is preventable, so avoid implying it is.

Questions for the consultation

- Is a filling, a build-up, an inlay/onlay or a crown more suitable for my tooth, and why?
- How will you keep the tooth dry during placement?
- What happens if the tooth needs more work later?
- How will you check it at follow-up?

Suggested headings and summary

- What is a composite build-up?
- When might your dentist suggest one?
- Composite, amalgam and other materials
- What happens at the visit
- Looking after the tooth afterwards
- How long does it last? (answer without a number)
- Questions to ask your dentist

Short summary: A composite build-up rebuilds part of a damaged tooth with tooth-coloured resin. Whether it suits you depends on how much of the tooth is left and where the damage sits, which your dentist can assess at an examination.

Search intent and terminology

Patients may write: tooth filling, white filling, tooth-coloured filling, chipped tooth repair, tooth bonding. Clinical terms: direct composite restoration, resin composite, core build-up. Use "build-up" once defined, and avoid repeating it for its own sake.

Local SEO note

Name the location only in a location block (for example, "Available at" with the verified address) once Neodent confirms which clinic provides this service. See the local search section.

Clinician-review flags

- How does Neodent define "composite build-up" versus a filling or a core build-up under a crown?
- Which clinic and which clinicians provide it.
- Any statement about visit count, anaesthesia or rubber dam use.

Sources

- ADA MouthHealthy: Composite Fillings, undated, © 2026 ADA
- ADA: Materials for Direct Restorations, updated 16 Aug 2024
- University of Bristol Dental School: Composite white fillings leaflet, undated

2. Dental implants

Page angle: a staged-journey page. Patients want to know how long it takes, what the stages are, what can stop treatment, and what long-term care involves. A timeline-style "stages" section and a "who may not be suitable yet" section fit this treatment better than a generic benefits list.

What it is

An implant is a titanium, screw-shaped post placed in the jawbone to support a crown, bridge or denture. It can replace one or more teeth or steady a loose denture (Cambridge University Hospitals patient information, approved 14 Jan 2025).

Why a dentist may discuss it

An implant may come up when a tooth is missing or cannot be saved, or when a denture is loose. The Royal College of Surgeons' Faculty of Dental Surgery also notes implants can hold prostheses after jaw surgery for cancer or trauma (RCS briefing, 23 Jul 2014, now dated). Conventional options such as bridges and dentures are alternatives and should be shown on the page.

Options and practical differences

Choice	What to explain
Single implant, several implants, implant-retained denture	Number of implants depends on how many teeth are missing and the type of restoration
One-stage or two-stage surgery	Two-stage needs a second minor procedure to uncover the implant (Cambridge)
Bone graft first	May add at least four months (Cambridge)
Bridge or denture instead	Different trade-offs, no surgery in the jaw

How suitability is assessed

Cambridge describes standard X-rays first, 3D CT scans in some cases, dental moulds for planning, and a check of bone height and volume. An Abu Dhabi Department of Health guideline (March 2026) lists a full medical and oral assessment, imaging such as OPG and CBCT where indicated, risk factors (diabetes, bruxism, smoking, gum disease history), informed consent and willingness to commit to long-term maintenance (DoH Abu Dhabi guideline). That is a regional regulator's guideline, not an Indian one, so use it as supporting evidence only. The right plan depends on individual assessment.

Typical journey (general information)

1. Consultation, examination, imaging and moulds or scans.
2. Any preparation first: gum disease treatment, grafting, stopping smoking.
3. Surgery to place the implant, with local anaesthetic.
4. Healing. Cambridge says three to six months for the implant to bond, longer after a graft.
5. Restorative stage: impression, then crown, bridge or denture attached.

Cambridge states total treatment usually takes at least a year. Do not quote this as Neodent's timeline until the dentist confirms.

Recovery and aftercare

Discomfort and swelling are normal for several days, with some discomfort for about a week, helped by simple painkillers (Cambridge). A review about a week after surgery checks healing. Follow the treating clinician's written instructions.

Risks and when to call

- Implant may not bond and may need removal.
- Infection if instructions are not followed.
- Nerve injury is rare. The RCS briefing reports lower-jaw nerve injury as a recognised risk.
- Long-term gum inflammation (mucositis) or bone loss (peri-implantitis) around the implant.
- Contact the clinic urgently for increasing swelling, fever, bleeding that will not stop, a loose implant or crown, or numbness that does not settle.

Where sources differ: Estimates of peri-implantitis vary widely, from about 15% of implants (Abu Dhabi guideline) to about one third of patients (RCS 2014). Success figures also vary: Cambridge gives 80 to 95% over 15 years in healthy mouths, the Abu Dhabi guideline gives about 97% over five years. Recommend the page quote no success rate unless Neodent's dentist supplies one with a source.

Durability and maintenance

Cambridge says cleaning must be meticulous and professional check-ups and cleaning every three to six months are needed. The Abu Dhabi guideline gives five to six months, adjusted to risk. Crowns, bridges and dentures wear and may need replacing. The Scottish Dental Clinical Effectiveness Programme lists periodontitis history, smoking, diabetes, poor maintenance and poorly designed or fitted restorations as risk factors for peri-implant disease (SDCEP, undated).

Prevention and habits

Treat gum disease before placement where possible, clean around the implant daily, attend recalls, and stop smoking. Cambridge says smokers have a much higher failure rate and should stop at least six months before treatment, so say "stopping smoking is strongly advised" and leave the period to the dentist.

Questions for the consultation

- Am I suitable now, or does anything need treating first?
- Do I need a scan or a bone graft?
- What are the alternatives, and what do they involve?
- Who does the surgery and who makes the crown?
- What follow-up and maintenance will you arrange, and how often?
- What happens if the implant does not bond?

Suggested headings and summary

- What is a dental implant?
- Who might be offered an implant?
- Implants, bridges and dentures
- The stages of treatment
- Healing and aftercare
- Risks to know about
- Keeping an implant healthy
- Questions to ask

Short summary: A dental implant is a small titanium post that supports a replacement tooth or denture. It is a staged treatment that depends on healthy gums and enough bone, and it needs long-term cleaning and check-ups. Your dentist can tell you whether it suits you.

Search intent and terminology

Patients: tooth implant, missing tooth replacement, implant cost (do not give a price), implant vs bridge, loose denture fix. Clinical: osseointegration, peri-implantitis, bone graft, CBCT. Define each clinical term once.

Local SEO note

Use an "Implant consultations at" block only for locations where Neodent confirms implant treatment and which clinician performs surgery and restoration. Avoid "same-day" or "painless" claims unless verified.

Clinician-review flags

- Who places and who restores implants at each location, and their qualifications.
- Whether CBCT is on site.
- Implant systems or brands used, if the page names any.
- Any figures on success, timelines or recall interval.

Sources

- Cambridge University Hospitals: Dental implants in restorative dentistry, approved 14 Jan 2025
- SDCEP: Risk factors for peri-implant disease, undated
- Royal College of Surgeons FDS briefing, 23 Jul 2014
- DoH Abu Dhabi: Dental implants guideline, March 2026

3. Root canal treatment

Page angle: a worried-reader page. Most visitors arrive with pain or fear, so lead with what the treatment is for and what the alternatives are, then walk through a visit, then say what to do if pain or swelling continues. Keep a calm tone and avoid promising a pain-free experience.

What it is

Root canal treatment treats inflammation or infection of the pulp, the soft tissue at the centre of a tooth. It can follow decay, a leaking filling or trauma (NHS Lothian patient information, v1.0, April 2023, review due April 2026). Mayo Clinic describes it as a way to save a badly damaged or infected tooth instead of removing it (Mayo Clinic, 5 Sep 2025).

Why a dentist may discuss it

Mayo lists toothache, gum swelling, sensitivity to hot or cold, a hole in the tooth and facial swelling as warning signs. These signs alone do not confirm that a root canal is needed. A dentist examines the tooth, takes X-rays and tests the response of the tooth before advising.

Options

- Root canal treatment, usually followed by a filling, onlay or crown.
- Re-treatment of a tooth that has had a root canal before (NHS Lothian).
- Removing the tooth, with or without replacing it using an implant, bridge or partial denture (Mayo).

Say plainly that leaving an infection untreated can cause an abscess and bone loss (Mayo).

How suitability is assessed

Examination, X-rays, and often tests of how the tooth responds. Neither source here describes the tests in detail, so ask the dentist what Neodent uses before listing them. Whether the tooth can be restored afterwards is part of the decision.

Typical visit (general information)

1. Local anaesthetic and a rubber dam around the tooth.
2. An opening is made to reach the pulp, and the infected pulp is removed.
3. The canals are cleaned, shaped and disinfected.

4. A temporary filling, sometimes with a medicated dressing if more than one visit is needed.
5. The canals are filled and the tooth is restored.

Visit counts differ: Mayo says usually one or two visits, NHS Lothian says one to three depending on the tooth and difficulty. Present this as a range and avoid a promise. Steps combine both sources.

Recovery and aftercare

Mayo says the tooth may be sensitive for a few days, over-the-counter pain relief may help, and to avoid chewing on the tooth until it is permanently restored. NHS Lothian says a permanent restoration is usually needed to protect the tooth and lower the risk of fracture, and the dentist usually reviews about a year later. General information only: follow your own dentist's instructions.

Risks and when to call

NHS Lothian lists pain or swelling afterwards, a perforation, a broken instrument in the canal, tooth or root fracture, canals that cannot be fully cleaned and an unsuccessful result, and says these are uncommon. Mention them briefly and proportionately. Advise contacting the clinic if pain or pressure lasts beyond a few days (Mayo), or if swelling, fever or a loose temporary filling appears.

Durability and follow-up

Mayo says a restored tooth can last a lifetime with good care, which is a strong claim. Do not repeat it as a promise. NHS Lothian gives no success rate. Results depend on the tooth, how quickly it is restored, and ongoing cleaning and check-ups.

Prevention habits

The pulp problems that lead to root canals often follow deep decay, cracks or injury. Daily fluoride brushing, cleaning between teeth, limiting sugary snacks, regular check-ups and a mouthguard for contact sports can reduce risk, but not all cases are preventable. Add a source for this advice from the ADA or an equivalent body before publishing (gap).

Questions for the consultation

- Does this tooth need a root canal or is there another option?
- How many visits do you expect, and what will it involve?
- What will the tooth need afterwards, and when?
- What are the chances it will need further treatment?

- Who should I call if pain returns?

Suggested headings and summary

- What is a root canal treatment?
- Signs you should get a tooth checked
- What happens at the visit
- After the treatment
- What if you do not treat it?
- Why the tooth usually needs a crown or filling
- Questions to ask

Short summary: Root canal treatment removes the infected or inflamed centre of a tooth so the tooth can often be kept. Your dentist can tell you after an examination and X-rays whether it is the right option.

Search intent and terminology

Patients: root canal, tooth nerve treatment, tooth abscess, severe toothache, does a root canal hurt. Clinical: endodontic treatment, pulpitis, pulp, apical periodontitis, re-treatment. Say "nerve" only once alongside "pulp", since the pulp holds nerves and blood vessels.

Local SEO note

Do not claim emergency or same-day availability on a location page unless Neodent confirms it and the opening hours. Mention endodontist or microscope only if verified.

Clinician-review flags

- Whether an endodontist, general dentist or both treat patients, and where.
- Use of rubber dam, microscope, rotary files or single-visit treatment.
- Typical number of visits and who provides the crown.
- Wording of the urgent-contact advice and the emergency number.

Sources

- Mayo Clinic: Root canal treatment, 5 Sep 2025
- NHS Lothian: Root canal treatment or re-treatment, April 2023 (review date passed, check for an update)
- Gap: add a dental professional body source (AAE or equivalent)

4. Braces and orthodontics

Page angle: a commitment-and-timeline page. Braces depend on the patient's daily habits more than most treatments here, so the page should cover the active phase, the retention phase, daily care and what happens when appointments are missed. Keep adults, teenagers and parents in mind as separate readers.

What it is

Orthodontic treatment moves teeth, usually with brackets bonded to the teeth and wires, in an active phase followed by a retention phase in which retainers hold the teeth in place (University Hospitals of Leicester leaflet, produced June 2022, review due June 2025).

Why someone may book an assessment

Crowding, gaps, or a bite that does not meet well are common reasons, but the sources collected do not list indications in detail. Say that appearance and function are both discussed and that the orthodontist decides whether treatment is advisable.

Options

Option	What the sources say
Fixed (metal) braces	The standard way to move teeth (East Kent leaflet)
Removable functional appliances, headgear	Used in some cases, often for younger patients to improve the bite (East Kent leaflet)
Elastics	Worn as instructed in some cases (Leicester, Bristol)
Clear aligners	Not covered by any authoritative patient source gathered here. Needs a source and a Neodent dentist's statement before inclusion
Retainers (removable or fixed)	Given after treatment (Bristol)

East Kent leaflet: Orthodontic treatment: what you need to know, undated.

How suitability is assessed

The sources gathered say the orthodontist discusses alternatives and that braces depend on the patient keeping teeth clean. East Kent says that if cleaning is not possible, accepting the teeth as they are is the only option. They do not list the records taken (such

as X-rays, photographs and moulds or scans), so describe records only after the Neodent orthodontist confirms them. Bristol notes good dental health is needed before treatment.

Typical stages

1. Assessment and a plan, including any teeth that need removing.
2. Fitting the appliance.
3. Adjustment visits. Leicester says about every eight weeks.
4. Removal and retainers.

Leicester says an average course is one to four years. East Kent says 18 to 24 months usually, with complex cases longer. Missed appointments and broken braces can lengthen treatment. Present a range and do not offer a Neodent timeline without confirmation.

Adjusting to braces and daily care

- Teeth may ache for a few days after fitting and after adjustments. British Orthodontic Society leaflet says sore for 3 to 5 days (BOS leaflet, 2006, old).
- Avoid hard, chewy and sticky foods that can break the brace (Leicester).
- Brush two to three times a day for two to three minutes, clean around brackets and under the wire with interdental brushes, and use fluoride toothpaste (UH Bristol: braces, retainers and dental health, undated; written for patients having jaw surgery, so use for the cleaning advice only).
- Wax can ease a sharp part until it is repaired (Bristol).

General information only: follow the treating orthodontist's instructions.

Risks and limits

- White or brown marks on enamel from poor cleaning, and gum problems (Leicester, BOS).
- Root shortening, usually a few millimetres, occasionally more significant (Leicester, BOS).
- Treatment may be stopped early if cleaning is not adequate (Bristol, East Kent).
- Teeth can drift after treatment (BOS, East Kent).
- Contact the clinic for a broken brace with sharp edges, a loose band, or swelling or severe pain.

Durability, retention and follow-up

Leicester and Bristol advise retainers for life, part-time. East Kent says retainers are recommended for life because teeth tend to move back. Ask the orthodontist to state Neodent's own retainer advice, and any charge for replacements.

Prevention habits

Fluoride toothpaste and mouthwash, limiting acidic, sugary and fizzy drinks, six-monthly dental check-ups (Bristol, East Kent). Do not suggest braces can be avoided by habits.

Questions for the consultation

- What is the problem with my bite or alignment, and what happens if I leave it?
- Which options suit me, and why?
- How long might treatment take, and how often will I visit?
- Will any teeth need to be removed?
- What retainer will I need, and for how long?
- Who looks after my brace in an emergency?

Suggested headings and summary

- What are braces and orthodontic treatment?
- Types of braces
- Is orthodontic treatment right for me?
- What happens at each stage
- Looking after your teeth with braces
- Retainers and keeping the result
- Questions to ask your orthodontist

Short summary: Braces gradually move teeth into a better position, then retainers help keep them there. The outcome depends on your assessment and on daily care, so an orthodontic consultation is the place to find what is suitable.

Search intent and terminology

Patients: braces, teeth straightening, crooked teeth, gap in teeth, overbite, braces for adults, clear braces. Clinical: orthodontics, malocclusion, retainers, aligners. Only use "aligners" once a source and clinician confirm Neodent offers them.

Local SEO note

Say "orthodontic consultations at [location]" only where confirmed, and name an orthodontist only if registered and confirmed. Avoid "affordable" or "painless".

Clinician-review flags

- Whether Neodent has an orthodontic specialist, and at which location.
- Which appliances are offered, including aligners and ceramic braces.
- Records taken at assessment.
- Retainer policy and replacement charges.
- No before-and-after photos without clearing the advertising rules (see local search section).

Sources

- University Hospitals of Leicester: Having braces fitted, June 2022 (review date passed)
- East Kent Hospitals: Orthodontic treatment, what you need to know, undated
- UH Bristol: Braces, retainers and dental health, undated
- British Orthodontic Society: risks leaflet, 2006 (archived, old)
- Gap: a current source on clear aligners and on orthodontic records

5. Smile design

Page angle: an options-and-expectations page. "Smile design" is a marketing and planning term, not a single procedure, so the page should explain it as a planned combination of treatments and spend most of its length on the choices (whitening, bonding, veneers, alignment) and on what is reversible and what is not. This is the page most exposed to advertising rules, so keep claims modest.

What it is

None of the authoritative sources gathered defines "smile design" as a single treatment. Provider pages describe it as planning changes to the shape, colour, size and alignment of teeth. Present it that way, as a planning conversation, and ask the Neodent dentist how the clinic uses the term. Do not copy provider wording as a clinical definition.

Why someone may ask about it

Chipped, stained, uneven, gapped or crooked teeth, or worn edges. Appearance is a valid reason to book, but the dentist should check health first. The ADA says existing decay or gum disease must be treated before veneers (ADA MouthHealthy: Veneers, undated, © 2026).

Options and practical differences

Option	What the sources say	Key trade-off
Whitening	Peroxide bleach lightens stains. Crowns, veneers and fillings do not whiten (ADA MouthHealthy: Teeth Whitening, undated)	Results vary; sensitivity and gum irritation are common and usually temporary
Composite bonding or composite veneers	May need less enamel removed, fewer visits, easier to repair; less resistant to stains and wear (ADA veneers)	May need repair or replacement sooner
Porcelain veneers	Strong, long-lasting, natural-looking, usually cost more (ADA veneers)	A small amount of enamel is removed, which is irreversible
Crowns	Cover the whole tooth (ADA veneers)	More tooth preparation
Orthodontic alignment	See the braces section	Time and retainers

How suitability is assessed

Examination of teeth and gums, treatment of any decay or gum disease first, and a check of the bite. The ADA notes grinding, clenching or a deep overbite may make veneers a poor choice. For whitening, Leeds Teaching Hospitals' leaflet describes it as suitable for teeth that have darkened after injury, root canal treatment or some medicines, and lists alternatives (no treatment, microabrasion, composite or porcelain veneers or crowns) (Leeds Teaching Hospitals, reviewed 4 Jun 2025). Photographs, moulds or digital scans and a trial or mock-up may be used, but no authoritative source gathered describes these, so ask the dentist.

Typical process

- Porcelain veneers: enamel is lightly reduced, an impression or 3D scan goes to a lab, a temporary may be worn, and the veneer is checked and bonded (ADA).
- Composite: the tooth is prepared, resin is sculpted, hardened with light, smoothed and polished (ADA).
- Whitening: Leeds describes custom trays and about three appointments over roughly two weeks for vital teeth. ADA describes in-office, dentist-supplied tray and over-the-counter routes.

Avoid giving visit counts for Neodent.

Adjustment and aftercare

The bite may feel different at first and the dentist should adjust it (ADA). Avoid biting nails and hard objects, limit staining drinks, and wear a mouthguard for contact sports (ADA). During whitening, Leeds advises avoiding strongly coloured foods such as curry, red wine and tomato-based foods. Follow the treating dentist's instructions.

Risks and limits

- Veneer preparation is irreversible (ADA).
- Veneers can chip, crack, wear or loosen, and decay can still form under or around them (ADA).
- Whitening results vary, can fade, and existing fillings or veneers will not change colour (Leeds, ADA).
- Sensitivity and gum irritation from whitening are usually temporary. Leeds lists root resorption as very rare.
- Cosmetic work done by unlicensed providers can cause tooth damage, nerve damage and infection (ADA News, 17 May 2024). A reassurance line about treatment by registered dentists is appropriate, but do not name a regulator without checking.

Durability and maintenance

The ADA veneers page gives no lifespan, and the Leeds leaflet says colour may darken again and need repeat whitening. Do not state a number of years. Brush twice daily with fluoride toothpaste, clean between teeth, and keep check-ups.

Prevention

Healthy gums and low decay risk keep cosmetic work in good condition. Not all discolouration is preventable (some follows injury or medicines).

Questions for the consultation

- Which option is least invasive for what I want?
- What is reversible and what is not?
- Will any of my existing fillings or crowns need replacing to match?
- Can I see a mock-up or plan before treatment?
- How will you adjust my bite, and what care will it need?

Suggested headings and summary

- What is smile design?
- Whitening, bonding, veneers and straightening compared
- Starting with a healthy mouth
- What is reversible and what is not
- Looking after your results
- Questions to ask

Short summary: Smile design is a planned approach to improving the look of your teeth, which may combine options such as whitening, bonding, veneers or straightening. Your dentist can explain which are suitable for your teeth and what each involves.

Search intent and terminology

Patients: smile makeover, teeth whitening, veneers, fix chipped teeth, close gaps. Clinical: aesthetic or cosmetic dentistry, direct bonding, porcelain veneers, bleaching. Avoid "perfect smile" and "Hollywood smile".

Advertising caution (high priority)

India's National Dental Commission, through its Ethics and Dental Registration Board, issued an advisory on 14 Oct 2026 cautioning registered dentists against, among other things, publishing patient testimonials and before-and-after photos, misleading treatment claims and self-aggrandising publicity, citing the Revised Dentists (Code of Ethics) Regulations, 2014 (Medical Dialogues report, 15 Oct 2026). Treat before-and-after galleries on this page as not publishable until Neodent's dentist or legal adviser confirms the current rules. This comes from a news report, so check the primary advisory.

Local SEO note

Smile design pages invite "best smile makeover in [area]" wording. Do not use it. Use factual wording such as "cosmetic dentistry consultations at [verified clinic name], [locality]" once the service is confirmed.

Clinician-review flags

- Definition of smile design at Neodent and what it includes.
- Whether veneers, whitening, aligners or digital design are done in-house.
- Any use of photos, mock-ups or testimonials.
- Lab and materials named on the page.

Sources

- ADA MouthHealthy: Veneers, undated
- ADA MouthHealthy: Teeth Whitening, undated
- Leeds Teaching Hospitals: Tooth whitening, reviewed 4 Jun 2025
- ADA News: veneer technicians, 17 May 2024
- Medical Dialogues: NDC advisory, 15 Oct 2026

6. Full mouth rehabilitation

Page angle: a "planned in phases" page for readers with several problems at once. Explain that it is a coordinated plan, not one procedure, then show how a plan is built. Be careful here: the strongest sources gathered are weaker than for the other treatments (see the evidence note), so the page should make fewer specific claims and rely on the Neodent dentist for detail.

Evidence note

No guideline or public-health patient page for "full mouth rehabilitation" was found. The sources below are a professional body's consumer page that also works as a find-a-dentist directory, a hospital group's service page, and a 2016 review of one cause of wear. None give timelines, risks or maintenance in detail. Use them for definition and components only, and take the rest from the Neodent dentist.

What it is

The American College of Prosthodontists' consumer page describes full mouth reconstruction as any dental treatment that affects all the teeth, combining restorative and cosmetic dentistry (ACP, Full Mouth Reconstruction, undated, © 2026). Aster Hospitals, an Indian hospital group, describes full mouth rehabilitation as a comprehensive plan to restore health, function and appearance of the whole mouth (Aster Hospitals, provider page, undated). "Rehabilitation" and "reconstruction" are used for the same idea. Choose one term and mention the other once.

Why a dentist may discuss it

The ACP page lists multiple missing teeth, failing large fillings, decay, cracked or broken teeth, and heavy wear from grinding. The Aster page adds significant bite problems and severe gum disease. A 2016 review in *Clinical, Cosmetic and Investigational Dentistry* says tooth wear is the most common clinical result of sleep bruxism, and that severe wear can reduce remaining tooth structure and change the bite height (Mengatto and colleagues, 2016). Symptoms like worn or sensitive teeth do not alone mean this treatment is needed.

Components that may be in a plan

Component	Examples named in the sources
Restorative	Onlays, crowns, bridges, veneers, implants, dentures
Gum health	Scaling and root planing, gum grafting (Aster)
Alignment	Braces or aligners where alignment contributes (ACP, Aster)
Bite	Occlusal adjustment (Aster); a splint, which the 2016 review calls useful for controlling bruxism
Cosmetic	Whitening, bonding, reshaping (Aster)

Cross-refer to the implant, braces, composite and smile design pages rather than repeat them.

How suitability is assessed

Aster describes review of medical and dental history with X-rays, digital scans and impressions, then an individual plan. The ACP says a consultation with a prosthodontist is required and a team often works together. The review says severe wear often needs multidisciplinary treatment and that increasing bite height is described as a key step in some cases. Ask the Neodent dentist whether Neodent has a prosthodontist and how cases are planned.

Typical journey (general information)

1. Detailed assessment and records.
2. A written plan that sets the order of treatment, with urgent problems and gum health usually first (Aster).
3. Phased treatment over several visits.

4. Review and long-term maintenance.

Aster says treatment often takes several months to over a year with multiple visits. This is a provider statement and should not be quoted as Neodent's timeline.

Recovery and aftercare

No source gathered gives aftercare for the whole programme. Recovery depends on which procedures are included, so refer to the relevant treatment pages and tell readers to follow written instructions from their treating dentist.

Risks and limits

- Each component carries its own risks (see implants, root canal and braces sections).
- The 2016 review says bruxism is linked to higher failure risk of restorations, with fracture as the main failure mode, and that implants in people who grind are controversial and decided case by case.
- The review also states that treatment relies largely on clinical experience rather than strong evidence, with no consensus on materials or techniques and few long-term studies.
- Large, long plans involve time, cost and commitment. Do not give prices.

Durability and maintenance

The review says restoration longevity depends on regular recalls and continued splint use in bruxism. Do not give a lifespan.

Prevention

Some causes can be reduced: gum disease and decay with daily cleaning and check-ups, and damage from grinding with a splint if the dentist advises. Congenital conditions, injury and some medical treatments cannot be prevented, and the ACP page lists some of these.

Questions for the consultation

- What is causing the problems, and will treating only the symptoms hold up?
- What does the full plan include, in what order, and with which clinicians?
- Can I do it in stages and stop between them?
- How will my bite be checked and protected afterwards?
- What will it involve in time and follow-up?
- What happens if one part of the plan fails?

Suggested headings and summary

- What is full mouth rehabilitation?
- When might it be discussed?
- What a plan can include
- How a plan is made
- Treating in stages
- Protecting the results
- Questions to ask

Short summary: Full mouth rehabilitation is a planned, step-by-step approach to restore the health, function and appearance of the teeth when many teeth are affected. What it includes depends on a detailed assessment by your dentist.

Search intent and terminology

Patients: full mouth restoration, rebuild all teeth, worn teeth treatment, many missing teeth, teeth grinding damage. Clinical: full mouth reconstruction, occlusal rehabilitation, prosthodontics, bruxism, tooth wear, vertical dimension. Keep clinical terms to a short explainer box.

Local SEO note

This page makes the strongest implied claim about clinic capability (multi-clinician, multi-stage). Do not say "specialist team", "prosthodontist" or "advanced" unless verified for the named location.

Clinician-review flags

- Whether Neodent does full mouth rehabilitation, and at which location.
- Which clinicians are involved and their qualifications.
- Which components are in-house and which are referred.
- The assessment records and the treatment-planning process.
- Replace weaker sources with a guideline or textbook reference suggested by the dentist.

Sources

- American College of Prosthodontists: Full Mouth Reconstruction, undated (consumer and directory page)
- Aster Hospitals: Full mouth rehabilitation, undated (provider page)

- Mengatto and colleagues: Sleep bruxism, challenges and restorative solutions, Clinical, Cosmetic and Investigational Dentistry, 22 Apr 2016

7. Scaling and polishing

Page angle: a short, reassuring routine-care page that is honest about what a clean does and does not do. The main job is to separate routine cleaning (scale and polish) from deeper gum treatment (scaling and root planing), because patients often confuse the two and the evidence for each is different.

What it is

A hygienist or dentist removes plaque and hard tartar (calculus) from the teeth and polishes them. A National Health Service hospital leaflet describes the visit as thorough cleaning that usually takes about 10 to 15 minutes, plus advice on brushing, cleaning between teeth and diet (Dorset County Hospital NHS leaflet, written March 2021, review date passed). Because that is a hospital leaflet for its own service, the Neodent dentist should confirm timing.

Why a dentist may discuss it

Visible tartar, bleeding gums, stained teeth, or bad breath can lead to a check. The ADA lists red, swollen, tender or easily bleeding gums, receding gums, persistent bad breath or taste and loose teeth as warning signs of gum disease, and notes gum disease can occur with no warning signs (ADA MouthHealthy: Gum Disease, undated, © 2026). Signs alone do not confirm the cause.

Options: routine cleaning or deeper treatment

	Routine scale and polish	Scaling and root planing
Purpose	Removes plaque, tartar and stain from tooth surfaces	Deeper cleaning below the gumline to treat gum disease
Who it is for	Gingivitis or healthy gums	Gum pockets too deep for a routine cleaning
Visits and comfort	Usually a short visit	May take more than one visit and may need local anaesthetic
Aftercare	Little needed	Soreness for a day or two, sensitivity up to about a week

Sources: ADA Gum Disease page and ADA MouthHealthy: Scaling and Root Planing (undated). The ADA says professional cleaning is the standard treatment for gingivitis, while treatment for periodontitis depends on the type and how far it has progressed.

How suitability is assessed

The sources gathered do not describe the exam in detail. The ADA scaling and root planing page says the dentist measures pocket depth and re-measures at follow-up. Say the dentist or hygienist checks the gums and decides whether a routine clean is enough, and ask Neodent to confirm what the assessment includes (probing, X-rays).

Typical visit

Removal of plaque and tartar, polishing, advice on home care, and sometimes fluoride varnish or antibacterial gel or rinse for adults with gum disease (Dorset leaflet). Ultrasonic or hand instruments are common but not described in these sources, so leave instrument types to the dentist.

Afterwards

The ADA says after scaling and root planing, expect pain for a day or two and sensitivity for up to a week, with gums that may be swollen, tender or bleed. Dorset's leaflet recommends brushing at least twice a day with fluoride toothpaste, spitting out rather than rinsing, and cleaning the tongue. Follow the treating clinician's instructions.

Risks and when to call

Sensitivity and bleeding are the usual effects after deeper cleaning. Contact the clinic for severe or increasing pain, swelling, fever, or bleeding that does not settle. The 2018 Cochrane review did not measure side effects such as sensitivity or surface damage, so do not claim cleaning is risk-free.

How often: where the evidence is limited

This is the key accuracy point for this page. A Cochrane review (Lamont and colleagues, published December 2018) found two UK trials with just over 1,700 adults without severe gum disease, and scheduled six or twelve-monthly scale and polish made little or no difference to gingivitis or probing depths compared with treatment only when the dentist judged it necessary, with high-certainty evidence (Cochrane news summary, 18 Mar 2019; Dental Elf summary, 11 Jan 2019). Scheduled cleaning slightly reduced tartar, and people felt their teeth were cleaner. A 2025 Dental Update review reached a similar view about

routine cleaning for prevention and noted repeated hygiene instruction gave similar plaque and bleeding benefits (Dental Update, 2025, vol 48). The evidence applies to low-risk adults and not to people with gum disease.

So the page should not say "everyone needs a clean every six months". Say the right interval depends on the individual's gum health and risk, and the dentist advises it.

Durability and prevention

A clean does not last. Results depend on daily brushing twice a day with fluoride toothpaste, cleaning between teeth daily, a balanced diet, avoiding tobacco and regular check-ups (ADA). The ADA lists smoking, diabetes, poor oral hygiene and some medicines as gum disease risk factors. Say that not every case is preventable.

Questions for the consultation

- Are my gums healthy, or do I need deeper treatment?
- How often should I come back, and why?
- What should I change in my brushing and cleaning between teeth?
- Will it be uncomfortable, and what can be done about sensitivity?

Suggested headings and summary

- What is scaling and polishing?
- Scale and polish or deeper gum cleaning?
- What happens at the appointment
- After your clean
- How often do you need one?
- Looking after your gums at home

Short summary: Scaling and polishing removes plaque, tartar and stains from your teeth. How often you need it and whether you need a deeper gum treatment depends on the health of your gums, which your dentist can check.

Search intent and terminology

Patients: teeth cleaning, dental cleaning, tartar removal, scale and polish, deep cleaning, bleeding gums. Clinical: scaling, root planing, calculus, gingivitis, periodontitis, oral prophylaxis. Teach the difference between "cleaning" and "deep cleaning" once.

Local SEO note

This is a high-volume, generic query type, but the sources do not support claims about local demand, so do not state any. Use plain location wording such as "teeth cleaning at [verified clinic], [locality]" and avoid "cheap" or "painless".

Clinician-review flags

- Who performs cleans (dentist or hygienist) and at which location.
- Whether Neodent does root planing or refers gum disease cases.
- The recall interval advice Neodent wants to give.
- Whether ultrasonic scalers, air polishing or fluoride varnish are offered.

Sources

- Dorset County Hospital NHS: Your visit to the dental hygienist, March 2021
- ADA MouthHealthy: Gum Disease, undated
- ADA MouthHealthy: Scaling and Root Planing, undated
- Cochrane news summary: routine scale and polish, 18 Mar 2019 (review Dec 2018)
- Dental Elf: Routine scale and polish, 11 Jan 2019
- Dental Update: routine scale and polish review, 2025

Cross-treatment source list

Only pages opened during this research are listed. "Undated" means no publication or review date was shown on the page.

Professional bodies and public-health or hospital sources

- ADA MouthHealthy: Composite Fillings, undated
- ADA: Materials for Direct Restorations, updated 16 Aug 2024
- ADA MouthHealthy: Veneers, undated
- ADA MouthHealthy: Teeth Whitening, undated
- ADA MouthHealthy: Gum Disease, undated
- ADA MouthHealthy: Scaling and Root Planing, undated
- ADA News: ADA advises public about veneer technicians, 17 May 2024
- American College of Prosthodontists: Full Mouth Reconstruction, undated (consumer and directory page)
- Royal College of Surgeons FDS: briefing on dental implants, 23 Jul 2014 (dated)
- SDCEP: Risk factors for peri-implant disease, undated

- DoH Abu Dhabi: Dental implants guideline, March 2026
- British Orthodontic Society: risks leaflet, 2006 (archived)

Hospital and university patient leaflets

- Cambridge University Hospitals: Dental implants in restorative dentistry, approved 14 Jan 2025
- NHS Lothian: Root canal treatment or re-treatment, April 2023 (review date passed)
- Mayo Clinic: Root canal treatment, 5 Sep 2025
- University Hospitals of Leicester: Having braces fitted, June 2022 (review date passed)
- East Kent Hospitals: Orthodontic treatment, what you need to know, undated
- UH Bristol: Braces, retainers and dental health, undated
- University of Bristol Dental School: Composite white fillings, undated
- Leeds Teaching Hospitals: Tooth whitening, reviewed 4 Jun 2025
- Dorset County Hospital NHS: Your visit to the dental hygienist, March 2021 (review date passed)

Research literature and evidence summaries

- Cochrane news summary: routine scale and polish, 18 Mar 2019 (review Dec 2018). The full review text could not be opened here.
- Dental Elf: Routine scale and polish for periodontal health, 11 Jan 2019
- Dental Update: routine scale and polish review, 2025, vol 48
- Mengatto and colleagues: Sleep bruxism, challenges and restorative solutions, 22 Apr 2016

Provider and news pages (not clinical authorities)

- Aster Hospitals: Full mouth rehabilitation, undated (provider page, used for definition and components only)
- Medical Dialogues: National Dental Commission warns dentists against unethical social media advertising, 15 Oct 2026 (news report, check the primary advisory)

Searched but not used

The American Association of Endodontists page reached covered only the office visit (last modified 4 Nov 2017) and was not cited. The Cochrane full text on PubMed Central could not be opened. Several clinic marketing pages appeared in searches and were not used as clinical support.

Editorial checklist

Run this for each treatment page before it goes to the dentist, then again before publishing.

Accuracy

- [ ] Every clinical claim traces to a source in this pack, or to the Neodent dentist in writing
- [ ] Source dates are checked again at publication and leaflets past their review date are replaced
- [ ] No price, success rate, guaranteed timeline, lifespan or "painless" wording
- [ ] Where sources disagree, the page says outcomes vary instead of choosing a number
- [ ] Symptoms are described as reasons to book an assessment, never as a diagnosis

Patient tone

- [ ] Plain language, with each clinical term explained once
- [ ] Risks stated proportionately, with when to contact the clinic urgently
- [ ] Recovery and aftercare labelled as general information, with "follow your dentist's instructions"
- [ ] A line that website information cannot determine what treatment an individual needs
- [ ] Alternatives, including no treatment, shown where relevant

Structure

- [ ] The page uses the structure suggested for that treatment, not a copied template
- [ ] Headings answer a patient's question and are not stuffed with keywords
- [ ] Questions to ask at a consultation are included

Local and compliance

- [ ] Only verified name, address, phone and hours, matching the Google Business Profile
- [ ] No service, clinician, equipment or availability claim for a location without written confirmation
- [ ] No "best", "leading", "near me" or service-area claims without evidence
- [ ] No before-and-after photos, testimonials or named reviews until the advertising rules are checked against the primary advisory
- [ ] Reviewing dentist's name and a "last reviewed" date on the page

Sign-off

- [ ] Dentist review complete for all flagged items
- [ ] Final read for broken links and contact details